

Policy on Credentialing and Privileging of Medical Professionals

«Hospital Name»

Document control

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Prepared by	HR In-Charge / Personnel Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» runs a process for credentialing and privileging of medical professionals permitted to provide patient care without supervision, so that only identified, qualified, and privileged professionals provide unsupervised care.

1. Purpose

This policy sets out how «Hospital Name» identifies medical professionals permitted to provide unsupervised patient care, documents and verifies their credentials, grants and communicates their privileges, and ensures they practise within those privileges.

This policy does not cover recruitment, induction, safety and quality training, or staff health and safety — those are covered in other hospital policies.

2. Scope

This policy applies to all medical professionals at «Hospital Name» permitted by law, regulation and the organisation to provide patient care without supervision.

3. Policy standards

«Hospital Name» identifies medical professionals permitted to provide patient care without supervision.

«Hospital Name» documents and periodically updates the education, registration, training and experience of identified medical professionals.

«Hospital Name» verifies information about medical professionals when possible.

«Hospital Name» grants medical professionals privileges to admit and care for patients in consonance with their qualification, training, experience and registration, reviewed at least annually.

«Hospital Name» ensures the requisite services a medical professional is authorised to provide are known to that professional and to relevant departments.

«Hospital Name» ensures medical professionals admit and care for patients as per their privileging.

4. Non-negotiable rules

1. Do not let a medical professional provide unsupervised patient care without first identifying them on the permitted list.
2. Do not leave a medical professional's education, registration, training or experience undocumented or out of date.
3. Verify every medical professional's credentials with the awarding organisation whenever possible.
4. Do not grant or allow privileges that are not in consonance with a medical professional's qualification, training, experience and registration.
5. Do not leave a medical professional or the relevant departments unaware of the services that professional is authorised to provide.
6. Do not let a medical professional admit or care for a patient outside their granted privileges.
7. Do not bypass the stop-work authority in Section 6 when the trigger conditions are met.
8. Staff who see a rule under this policy broken report it the same shift to the HR In-Charge / Personnel Officer or the Medical Superintendent.

5. What we do

5.1 Identification of permitted professionals

«Hospital Name» identifies medical professionals permitted by law, regulation and the organisation to provide patient care without supervision — individuals with the required qualification(s), training and experience, in consonance with the law. Providing unsupervised care outside this identified list is a stop-work trigger (Section 6).

5.2 Credential documentation

The education, registration, training and experience of identified medical professionals at «Hospital Name» are documented and updated periodically — after acquisition of new skills or qualification — and maintained in each professional's personal file.

5.3 Credential verification

Information about medical professionals at «Hospital Name» is appropriately verified when possible, by checking with the organisation that awarded the qualification or training. The National Medical Commission's website is a useful reference for verification.

5.4 Granting of privileges

«Hospital Name» grants medical professionals privileges to admit and care for patients in consonance with their qualification, training, experience and registration — identifying the clinical services each is authorised to perform (for example, radiotherapy only by a radiation oncologist). Privileges are reviewed at least annually and revised where necessary. Granting or exercising privileges outside this record is a stop-work trigger (Section 6).

5.5 Communication of privileges

The requisite services a medical professional at «Hospital Name» is authorised to provide are known to that professional and to the relevant departments — for example OP consultation rights, admission rights and rights to specific procedures or surgeries (inclusion or exclusion). Concerned departments (for example, front desk for admission rights, the operation theatre for surgical rights) are informed of the relevant privileging.

5.6 Practising within privileges

Medical professionals at «Hospital Name» admit and care for patients as per their privileging, using a standardised format applied uniformly. New faculty may work under proctorship until independent privileges are granted; the hospital maintains a mechanism confirming professionals provide only the services they are privileged for.

6. Stop-work authority

Do not let a medical professional provide patient care without supervision unless the HR In-Charge / Personnel Officer and the credentialing file show that professional identified and privileged for that care under this policy.

Stop-work applies to starting or continuing unsupervised care by that professional. Care already under way is handed to a privileged professional or brought under supervision; immediate life-saving care is not withdrawn while that handover happens.

The person who stops tells the HR In-Charge / Personnel Officer and the Medical Superintendent the same shift. Refusing to let an uncredentialed or unprivileged professional practise unsupervised is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
HR In-Charge / Personnel Officer	Owns this policy. Maintains the credentialing file and the identified-professional list.
Credentialing Committee	Reviews credentials and grants privileges;

	conducts the annual privilege review.
Medical Superintendent	Oversees implementation of this policy. Receives stop-work escalations.
All Staff	Report any medical professional providing unsupervised care outside the identified or privileged list.

8. Quality monitoring

The HR In-Charge / Personnel Officer reviews credentialing files to confirm every medical professional providing unsupervised care is identified and privileged. Documentary evidence is on file for each CORE objective element. Compliance with the stop-work authority in Section 6 is monitored through the credentialing file review. This policy itself is reviewed every year.

9. Training and staff acknowledgement

All staff covered by this policy are made aware of the credentialing and privileging process, including the stop-work authority in Section 6, and sign the acknowledgement below.

I have read the Policy on Credentialing and Privileging of Medical Professionals of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

HR department; Medical Superintendent's office; all department heads / unit in-charges; all staff through the hospital intranet and department policy folders.

11. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
HR	Human Resources
HRM	Human Resource Management (NABH Hospitals 6th Edition chapter)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element

12. Traceability to NABH HCO Full Accreditation 6th Edition HRM.11

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required. Stop-work applies ONLY to the OEs marked 'Section 6 Stop-work' below.

OE	Level	Requirement	Where addressed	Responsible
HRM.11.a	CORE	Medical professionals permitted by law, regulation and the organisation provide patient care without supervision are identified.	Section 3; 5.1; Section 6 Stop-work	HR In-Charge / Personnel Officer
HRM.11.b	Commitment	The education, registration, training and experience of the identified medical professionals are documented and updated periodically.	Section 3; 5.2	HR In-Charge / Personnel Officer
HRM.11.c	Commitment	The information about medical professionals is appropriately verified when possible.	Section 3; 5.3	HR In-Charge / Personnel Officer
HRM.11.d	CORE	Medical professionals are granted privileges to admit and care for patients in consonance with their qualification, training, experience and registration.	Section 3; 5.4; Section 6 Stop-work	HR In-Charge / Personnel Officer
HRM.11.e	Commitment	The requisite services to be provided by the medical professionals are known to them as well as the various departments / units of the organisation.	Section 3; 5.5	HR In-Charge / Personnel Officer
HRM.11.f	Commitment	Medical professionals admit and care for patients as per their privileging.	Section 3; 5.6	HR In-Charge / Personnel Officer

13. Required Records / Evidence Checklist

HRM.11.a — Permitted medical professionals identified. [Stop-work trigger]

- Identified-medical-professional list permitted to provide unsupervised patient care, with a qualification, training and experience verification record.
- Cross-reference to the stop-work authority in Section 6 for unlisted practice.

HRM.11.b — Credentials documented and updated periodically.

- Education, registration, training and experience documentation record, updated periodically, with a personal-file record of each update.

HRM.11.c — Credentials verified when possible.

- Verification record with the awarding organisation, including a National Medical Commission or equivalent reference-check record.

HRM.11.d — Privileges granted in consonance with qualification, training, experience and registration. [Stop-work trigger]

- Granted-privilege record naming the clinical services each professional is authorised for, with an annual privilege-review record.
- Cross-reference to the stop-work authority in Section 6 for privileging outside this record.

HRM.11.e — Requisite services known to professional and relevant departments.

- Communicated-service record to the professional and to relevant departments, including admission-rights or surgical-rights notification records (front desk, operation theatre, etc.).

HRM.11.f — Professionals admit and care as per privileging.

- Standardised privileging-format record, applied uniformly.
- Proctorship record for new faculty until independent privileges are granted.
- Mechanism-confirmation record that professionals provide only privileged services.

14. References

- NABH Accreditation Standards for Hospitals, 6th Edition — standard HRM.11.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — HRM.11 interpretations.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

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